

NIC CPD Micro-Credential: Dementia Care, Communication & Behaviour Support

Issuing organisation: National Institute of Caregivers Nigeria (NIC)

Credential type: Stand-alone Continuing Professional Development (CPD) Micro-Credential

Recommended duration: 5–6 CPD learning hours

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Scope note: This is professional development for caregivers and direct-care workers who already understand basic caregiving. It is not an introductory dementia course, a full dementia-care qualification, a clinical dementia programme, a diagnostic course, a psychotherapy qualification or a specialist neurological-care credential. It does not make a learner a nurse, doctor, clinician, dementia specialist or safeguarding investigator.

1. Course title

NIC CPD Micro-Credential: Dementia Care, Communication & Behaviour Support

2. Course overview

This micro-credential helps working caregivers respond more effectively to the everyday challenges associated with dementia. It focuses on communication, confusion, disorientation, distress, behaviour as communication, resistance to care, agitation, repetitive behaviour, wandering, environmental triggers, independence, dignity, family communication, documentation and escalation.

Dementia describes a range of conditions and experiences affecting memory, thinking, communication, mood or daily functioning. It is not an inevitable part of ageing, and people living with dementia do not all experience the same symptoms, progression or level of dependence ¹. A person may remain able to make choices, enjoy relationships, communicate preferences, contribute to meaningful activity and experience a rich emotional life.

The course therefore uses the principle:

See the person, not only the dementia.

The central professional sequence is:

PAUSE → OBSERVE → CONSIDER THE PERSON → IDENTIFY POSSIBLE TRIGGERS → COMMUNICATE CALMLY → ADAPT THE APPROACH → PROTECT DIGNITY & SAFETY → DOCUMENT → ESCALATE WHEN NECESSARY.

The course treats behaviour as potentially meaningful communication. Agitation, pacing, repetitive questions, resistance, withdrawal or shouting may reflect pain, fear, fatigue, hunger, thirst, toileting needs, loneliness, overstimulation, unfamiliar surroundings, communication difficulty, medication effects, infection, delirium or another unmet need. The caregiver does not diagnose the cause. The caregiver observes, responds respectfully, records what happened and escalates changes that require professional assessment.

Course at a glance

Element	Specification
Target learners	Professional caregivers, healthcare assistants, home-care workers, residential-care staff, care supervisors, community-care workers and other direct-support professionals
Course type	Stand-alone CPD micro-credential
Learning time	5–6 CPD hours
Structure	6 modules, 18 lessons, module assessments and a 35-question final assessment
Learning emphasis	Personhood, communication, behaviour support, distress, resistance, safety, families, documentation and escalation
Recommended pass mark	80% (28/35)
Application focus	At least 25 of 35 final questions are scenario/application based
Practical limitation	Online completion demonstrates learning and scenario reasoning; it does not prove real-world behavioural competence
Credential limitation	Not a diagnosis, clinical qualification or specialist dementia-care licence

3. Strategic purpose of the CPD

Caregivers are often the people who notice the first meaningful change in a person's day: a meal left untouched, a new fear of bathing, repeated attempts to leave, a sudden change in speech, a new pattern of calling out or a client who is unusually sleepy. These observations can be clinically and emotionally important, yet a response based only on control, correction or routine can increase distress and undermine trust.

This CPD develops practical judgement. It helps learners interpret behaviour in context, adapt communication, identify possible triggers, preserve remaining abilities and support safe independence. It also makes clear when a change may be sudden or serious enough to require prompt clinical or supervisory review. NICE recommends structured assessment of distress to explore clinical, environmental, medication, communication and sensory causes before treatment is considered ³.

The credential complements NIC's Elderly Care & Gerontology programme by concentrating on dementia-specific communication and behaviour support rather than reteaching general ageing, basic personal care or introductory dementia facts. It is designed for immediate workplace application in Nigerian homes, residential settings, community services and healthcare-support environments.

The curriculum respects family involvement, faith, language, multigenerational households and community relationships. It does not assume that a family member always speaks for the person, that a culturally familiar approach is automatically safe, or that dementia removes autonomy. Where current Nigerian law, employer policy, clinical guidance or local reporting requirements are more specific, those requirements take precedence and should be inserted by NIC or the implementing employer before portal publication.

4. Target audience

The primary audience is working caregivers and direct-care workers who already understand dignity, respect, basic communication, personal care, safety, observation, reporting and professional boundaries. The course is suitable for home-care workers, residential-care assistants, healthcare assistants, community-care workers, care supervisors and family-support professionals working within a defined role.

It is not intended to train learners to diagnose dementia or delirium, prescribe or administer treatment outside authorisation, conduct a neuropsychological assessment, provide psychotherapy, make formal capacity determinations, investigate safeguarding allegations, initiate restraint or change a clinical care plan independently.

5. Recommended CPD hours

NIC should publish **5–6 CPD learning hours**. A practical allocation is approximately 3.5–4 hours for lessons, 60–75 minutes for lesson and module activities, and 45–60 minutes for

the final assessment, review and completion record. Portal credit should be based on meaningful interaction, assessment attempts and acknowledgement of scope rather than time spent with a page open.

6. Prerequisites

The recommended prerequisite is prior caregiving knowledge or experience. NIC may accept completion of a foundational caregiver course, its Elderly Care & Gerontology programme, another recognised caregiver programme or evidence of current direct-care work. Learners without basic caregiving knowledge should be directed to foundational education before attempting this CPD.

7. Learning outcomes

By the end of the CPD, the learner should be able to:

1. **Explain** why dementia is not a single experience and describe the importance of personhood, individuality and remaining abilities.
2. **Use** communication approaches that support understanding, choice, emotional safety and connection.
3. **Interpret** behaviour as possible communication of a need, feeling, discomfort or environmental response without treating an interpretation as a diagnosis.
4. **Identify** possible physical, emotional, environmental, communication, sensory and routine-related triggers for distress.
5. **Respond** to agitation, resistance, repetitive behaviour, confusion and disorientation with calm, proportionate and dignity-preserving strategies.
6. **Support** independence, meaningful activity, autonomy and choice while recognising safety limits and escalation needs.
7. **Recognise** wandering, getting lost, falls, aggression, sudden change and other safety concerns, and escalate appropriately.
8. **Communicate** effectively with families and colleagues while protecting confidentiality and avoiding blame.
9. **Document** observations, triggers, responses, outcomes and changes objectively and in a way that supports continuity.
10. **Apply** the NIC professional response framework: pause, observe, consider the person, identify triggers, communicate calmly, adapt, protect dignity and safety, document and escalate.

8. Competency framework

Competency domain	Expected capability	Evidence in the course
Personhood and dementia understanding	Describes variation, avoids stigma and identifies remaining strengths	Modules 1–2
Communication	Uses short, respectful, appropriately paced verbal and non-verbal communication	Module 2
Behaviour support	Treats behaviour as information and tests possible triggers without diagnosing	Modules 3–4
Distress and resistance	Uses de-escalation, choice, timing and environmental adaptation	Module 4
Independence and dignity	Supports autonomy and meaningful engagement while reducing unnecessary restriction	Modules 1, 4 and 5
Safety and escalation	Recognises wandering, falls, aggression, sudden change and possible delirium or illness	Module 5
Family partnership	Shares observations respectfully, listens to family knowledge and avoids blame	Module 6
Documentation and accountability	Records facts, patterns, triggers, responses and escalation accurately	Module 6
Professional scope	Does not diagnose, prescribe, investigate or use restraint outside authorisation	All modules

9. Professional response framework

The NIC P-O-C-T-A-P-D-E sequence

P — Pause. Stop automatic correction or rushed physical intervention. Regulate your own tone and body language.

O — Observe. Notice what happened before, during and after the behaviour. Look at words, facial expression, movement, pain cues, environment, people present and time of day.

C — Consider the person. Ask what the person may be experiencing, communicating, remembering, fearing or trying to achieve. Use the person's history, preferences, communication style and current abilities.

T — Trace possible triggers. Consider pain, toileting, hunger, thirst, fatigue, infection, constipation, medication effects, sensory loss, noise, crowding, temperature, unfamiliarity, rushed care, loss of control and emotional needs. Do not diagnose; identify what should be reported or checked.

A — Approach calmly. Use a gentle tone, simple language, one idea at a time, appropriate distance, eye contact if welcomed and extra time. Reduce competing voices and avoid arguing.

P — Personalise and adapt. Change the timing, environment, task steps, activity, communication method or offer of choice. Use familiar routines and meaningful interests where appropriate.

D — Dignity and safety. Protect privacy, avoid humiliation, use the least restrictive safe response and seek help when there is risk of injury, abuse, wandering or sudden deterioration.

E — Enter, escalate and evaluate. Document the observation, response and outcome. Escalate sudden, severe, repeated or unexplained changes and confirm that the concern reached the appropriate person.

The framework is a thinking aid, not a clinical diagnostic tool. A caregiver observes and reports; authorised clinicians assess and diagnose.

10. Complete module structure

Module	Title	Capability developed	Lessons	Assessment
1	Understanding the Person Living with Dementia	Sees the person beyond diagnosis and recognises variation, identity and	1. Dementia Is Not a Single Experience; 2. Personhood, History and Strengths; 3.	6 questions

		remaining abilities	Dignity, Autonomy and Meaningful Engagement	
2	Communication That Supports Connection	Adapts verbal, non-verbal and environmental communication	4. Foundations of Dementia Communication; 5. Confusion, Disorientation and Reassurance; 6. Family and Cultural Communication	6 questions
3	Behaviour as Communication	Identifies possible meaning, triggers and patterns without labelling the person	7. Reading Behaviour as Information; 8. Repetitive Behaviour, Calling Out and Withdrawal; 9. Observation and Trigger Mapping	6 questions
4	Distress, Agitation and Resistance to Care	Uses de-escalation, choice, timing and person-centred alternatives	10. Agitation and Distress; 11. Resistance to Personal Care; 12. De-escalation and Non-Pharmacological Support	7 questions
5	Safety, Wandering and Sudden Change	Balances independence with safety and recognises escalation needs	13. Wandering, Getting Lost and Environmental Safety; 14. Sundowning, Falls and Time-of-Day Changes; 15. Sudden Changes, Delirium Concerns and Escalation	7 questions

6	Partnership, Documentation and Professional Response	Communicates with families, documents objectively and integrates the framework	16. Family Partnership and Care Planning; 17. Documentation, Handover and Reporting; 18. Integrated Dementia-Care Judgement	7 questions
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11. Detailed lesson outline

Lesson	Purpose	Core application
1	Establish dementia as varied and individual	Avoid stereotypes and recognise different patterns and progression
2	Build personhood knowledge	Use life history, preferences, strengths and familiar identity
3	Protect dignity and autonomy	Support choice, independence and meaningful engagement
4	Adapt communication	Use pace, language, body language and sensory support
5	Respond to confusion and disorientation	Reassure, orient gently and avoid arguing
6	Work with family and culture	Share observations, invite knowledge and protect the person's voice
7	Reframe behaviour	Ask what the person may be communicating
8	Support repetitive behaviour and withdrawal	Identify function, provide reassurance and reduce distress
9	Map triggers	Record antecedent, behaviour, response and outcome
10	Understand agitation and distress	Check clinical and environmental contributors and de-escalate

11	Respond to resistance to personal care	Adapt timing, explanation, choice, staffing and task sequence
12	Use non-pharmacological support	Personalise activity, environment and emotional support; escalate clinical need
13	Manage wandering and getting lost	Preserve movement and independence while planning safety
14	Address sundowning, falls and time patterns	Adapt routine, lighting, fatigue management and supervision
15	Recognise sudden change	Treat acute deterioration as requiring prompt professional review
16	Communicate with families	Build partnership without blame, gossip or unauthorised disclosure
17	Document and report	Produce clear, objective records and effective handover
18	Integrate judgement	Apply P-O-C-T-A-P-D-E to complex scenarios

12. Full lesson content

Module 1 — Understanding the Person Living with Dementia

Lesson 1. Dementia Is Not a Single Experience

Why this matters in professional caregiving

Dementia is an umbrella term for a range of conditions that affect the brain. Alzheimer disease is the most common form, but vascular dementia, dementia with Lewy bodies, frontotemporal dementia and mixed forms can present differently ¹. People vary in age, language, education, personality, culture, health, sensory ability, life history and support

needs. A caregiver who expects one predictable pattern may miss strengths, misread communication or respond to a person as a diagnosis rather than an individual.

Learning objectives

The learner should be able to describe variation in dementia; challenge common stereotypes; identify why individual knowledge matters; and explain the limits of the caregiver role.

Core instructional content

Dementia can affect memory, thinking, communication, mood, behaviour and daily activities, but not necessarily in the same order or with the same intensity. One person may remember music and long-term experiences while struggling with recent instructions. Another may have fluent speech but difficulty judging risk. A person may be independent in some activities and need substantial support in others. Progression can be gradual, uneven or affected by other health problems.

Avoid assumptions such as “everyone with dementia wanders,” “aggression is inevitable,” “the person has lost their personality,” or “they cannot make any decisions.” Such statements are harmful because they encourage staff to stop asking, stop offering choice or treat distress as a problem to suppress.

Dementia is not the same as ordinary ageing. Forgetfulness can occur with age, but new or worsening confusion, functional decline or sudden change requires appropriate professional attention. Caregivers should not diagnose, but they should notice and report changes from the person’s baseline.

The caregiver’s role is to support daily life within the care plan, communicate respectfully, observe and record changes, protect safety and escalate concerns. It is not to label a person’s dementia type, explain every behaviour as dementia, recommend medication or assume that a change is “just the condition.”

Applying This in Practice

In home care, ask how the person usually communicates and what activities remain meaningful. In residential care, avoid comparing residents as though one progression is the standard for another. In healthcare-support settings, report a change from baseline rather than attributing it automatically to dementia. In community work, consider language, hearing, vision and unfamiliar environments.

Case scenario

A client who has dementia is usually able to dress with prompts. One morning, the client cannot follow a familiar sequence, appears unusually sleepy and struggles to speak. A

colleague says, “That is just the dementia.”

Correct professional response: Do not assume. Observe, protect immediate safety, document the change and escalate promptly to the responsible professional because a sudden change may have a physical or acute cause requiring assessment.

Common mistakes and professional risks

Common risks include stereotyping, treating a diagnosis as a complete description, assuming all distress is behavioural, ignoring strengths, using infantilising language and failing to report sudden change.

Key practice points

1. Dementia is varied; the person is not predictable from a label alone.
2. Do not assume aggression, wandering, incontinence or total dependence.
3. Notice changes from the person’s usual baseline.
4. Support abilities rather than taking over automatically.
5. Observe and report; do not diagnose.

Lesson assessment

1. **MCQ:** Dementia is best described as: A) one identical disease; B) a range of conditions and experiences affecting cognition and daily life; C) inevitable ageing; D) intentional difficult behaviour. **Answer: B. Objective tested: 1.**
2. **True/False:** Every person with dementia progresses at the same rate. **Answer: False. Objective tested: 1.**
3. **Scenario:** A sudden change is called “just dementia.” What should the caregiver do? **Answer:** Observe, document and escalate for professional review. **Objective tested: 3–4.**
4. **What should the caregiver NOT do?** Diagnose the dementia type based on one behaviour. **Answer:** Diagnosis is outside role. **Objective tested: 4.**
5. **Best next action:** A client can still dress with prompts. **Answer:** Support the remaining ability rather than taking over automatically. **Objective tested: 2–3.**
6. **Application:** Why are stereotypes unsafe? **Answer:** They can lead to missed needs, unnecessary restriction and loss of dignity. **Objective tested: 1–3.**

Lesson 2. Personhood, History and Strengths

Why this matters in professional caregiving

Person-centred dementia care begins with knowing the person beyond the diagnosis. Names, relationships, occupations, faith, language, food, routines, music, responsibilities, fears, achievements and preferred ways of receiving help can make communication and engagement more successful. Life history is not a decorative biography; it is a practical care resource.

Learning objectives

The learner should be able to gather and use personhood information; distinguish a preference from an assumption; identify remaining abilities; and use life history to personalise care without exposing private information unnecessarily.

Core instructional content

A person-centred profile should record what matters to the person, how they prefer to be addressed, important relationships, routines, cultural and religious preferences, likes and dislikes, meaningful activities, communication style, sensory needs, sources of comfort and signs of distress. Information should be gathered respectfully from the person first where possible, then from authorised family or supporters and the existing care plan.

The profile should not reduce the person to a list of deficits. Record what the person can do, what prompts help, what choices they make, what they enjoy and how they show “yes,” “no,” discomfort or interest. A person who cannot manage a full shower may still choose clothes, wash their face, fold towels or decide the order of tasks.

Use life history to connect, not manipulate. Music, prayer, familiar objects, food, language and household routines may support comfort, but the caregiver should not assume that an older person likes a particular song, religion or family role. Ask, observe and update.

Respect confidentiality. A life-history document should contain only relevant information, be stored securely and not be discussed as entertainment. The person’s story belongs to them. Avoid public storytelling, jokes about past work or sharing photographs without authorisation.

Applying This in Practice

In Nigeria, life history may include language, ethnic identity, community role, faith, extended-family relationships and migration history. Use these as sources of connection only with the person’s consent and preferences. A family’s account can help, but it should not erase the person’s current wishes.

Case scenario

A client becomes calmer when spoken to in a familiar language and when given time to arrange personal clothing. A new caregiver rushes the client and speaks only in formal

English.

Correct professional response: Use the person's preferred communication method where possible, preserve the familiar routine and document the approach that supports comfort and participation.

Common mistakes and professional risks

Risks include relying on stereotypes, assuming family preferences are the person's preferences, using private history casually, focusing only on deficits and treating life-story information as a script rather than a guide.

Key practice points

1. Know the person, not only the diagnosis.
2. Record strengths, preferences, routines and communication signals.
3. Ask rather than assume.
4. Use life history to support connection, not control.
5. Protect the person's story and privacy.

Lesson assessment

1. **Scenario:** A family says the client always liked a particular food, but the client now repeatedly refuses it. **Answer:** Respect the current preference, explore context and update the care information. **Objective tested: 2–3.**
2. **MCQ:** A person-centred profile should include: A) only deficits; B) strengths, preferences, routines and communication cues; C) gossip; D) unverified diagnoses. **Answer: B. Objective tested: 1.**
3. **True/False:** A past preference must always override a current refusal. **Answer: False. Objective tested: 2–3.**
4. **What should the caregiver NOT do?** Share a client's life story for entertainment. **Answer:** It breaches dignity and confidentiality. **Objective tested: 5.**
5. **Best next action:** A person communicates comfort through a familiar language. **Answer:** Use that language or authorised support where possible and document the effective approach. **Objective tested: 1–3.**
6. **Application:** Why record abilities? **Answer:** To support independence and avoid unnecessary takeover. **Objective tested: 3.**

Lesson 3. Dignity, Autonomy and Meaningful Engagement

Why this matters in professional caregiving

Dementia can change how a person completes tasks, but it does not remove the need for dignity, choice, purpose and belonging. Over-helping may be presented as kindness while quietly taking away independence. Meaningful engagement can support mood, identity, movement, connection and daily structure.

Learning objectives

The learner should be able to support autonomy; offer achievable choices; adapt tasks to remaining abilities; identify meaningful engagement; and avoid unnecessary restriction or infantilisation.

Core instructional content

Offer choices that are real and manageable: “Would you like the blue shirt or the white one?” “Would you like to wash now or after tea?” Avoid asking open questions that create unnecessary difficulty when two concrete options would help. Give time to respond. A choice should not be used to disguise coercion; do not offer options when the caregiver will reject the answer.

Break tasks into small steps. Lay out clothing in sequence, demonstrate one action, use visual cues, allow extra time and praise effort without sounding patronising. Support the person to do what they can. Independence is not measured by doing everything alone; it includes participation and influence.

Meaningful engagement is different from keeping someone busy. Activities should connect to the person’s interests, identity or current abilities. They may include music, prayer, conversation, gardening, cooking preparation, folding clothes, storytelling, walking, familiar household tasks, art, dance, community contact or quiet companionship. Consent and enjoyment matter.

Dignity requires privacy during personal care, respectful language, appropriate clothing, safe positioning and attention to the person’s emotional experience. Do not discuss the person as if they are absent. Do not call an adult “baby,” “naughty” or “difficult.” Describe the situation and need instead.

Applying This in Practice

In home care, preserve the person’s role in household routines where safe. In residential care, offer meaningful activities that are not childish or identical for everyone. In community settings, consider familiar places and safe social contact. In all settings, balance choice with the care plan and escalate when safety is affected.

Case scenario

A caregiver completes dressing quickly while the client watches. The client can select clothes and put on a shirt with prompts, but the caregiver says there is no time.

Correct professional response: Reorganise the task where possible to support participation, offer choices, allow time and report staffing or schedule problems that repeatedly remove independence.

Common mistakes and professional risks

Risks include taking over, using childish activities or language, offering false choices, confusing quiet compliance with wellbeing, restricting movement for convenience and ignoring the person's interests.

Key practice points

1. Support participation rather than automatic takeover.
2. Offer concrete, genuine choices.
3. Meaningful activity connects to identity and interest.
4. Dignity is protected in language, privacy and pace.
5. Convenience is not a sufficient reason to remove autonomy.

Lesson assessment

1. **MCQ:** A good choice is: A) genuine and manageable; B) a threat disguised as a choice; C) impossible to answer; D) decided by the caregiver. **Answer: A. Objective tested: 3.**
2. **Scenario:** The client can fold clothes and enjoys it. **Answer:** Include the activity where safe and meaningful. **Objective tested: 3–4.**
3. **True/False:** Adult clients with dementia should be spoken to like children because it is simpler. **Answer: False. Objective tested: 3.**
4. **What should the caregiver NOT do?** Restrict movement only because supervision is inconvenient. **Answer:** Seek a proportionate safety plan. **Objective tested: 3–5.**
5. **Application:** Give two ways to support dressing. **Answer:** Lay clothing in sequence; offer two choices; prompt one step at a time; allow time. **Objective tested: 3.**
6. **Scenario:** A client refuses an activity. **Answer:** Respect the refusal, explore another option later and do not force participation. **Objective tested: 3–4.**

Module 2 — Communication That Supports Connection

Lesson 4. Foundations of Dementia Communication

Why this matters in professional caregiving

Communication is more than the words used. Tone, pace, facial expression, distance, touch, lighting, hearing, background noise and the caregiver's emotional state all affect how a person with dementia experiences an interaction. Communication difficulties can lead to frustration, withdrawal and distress; adapted communication can preserve relationships and participation 4 .

Learning objectives

The learner should be able to use clear language, appropriate pace, supportive non-verbal communication, sensory adaptations and active listening.

Core instructional content

Approach from the front, identify yourself and use the person's preferred name. Make sure hearing aids, glasses or other communication supports are available where appropriate. Reduce background noise. Sit or stand at a respectful level, maintain a calm expression and avoid crowding.

Use short sentences with one idea at a time. Ask one question, then wait. Avoid rapid-fire questions, long explanations, sarcasm, idioms and arguing about details the person cannot recall. Offer visual or gesture-based support where helpful. Check understanding through the person's response rather than demanding repetition.

Non-verbal communication may carry more meaning as speech changes. Notice posture, breathing, facial expression, movement, gaze, hand gestures and withdrawal. Touch should be anticipated, welcomed and appropriate; do not grab from behind or use touch to force compliance.

Active listening means attending to what the person is expressing, acknowledging emotion, reflecting key words and checking what would help. If a person says, "I need to go home," respond to the feeling or purpose rather than correcting immediately: "You want to feel safe at home. Tell me what you are looking for."

Avoid testing memory. Questions such as "Do you remember me?" can create embarrassment. Introduce yourself naturally. Avoid correcting every error unless the error creates immediate risk. The goal is connection and safe understanding, not winning an argument.

Applying This in Practice

Use the person's preferred language and authorised interpreter or communication support where needed. In a busy Nigerian household, reduce the number of people

speaking at once. In residential care, consistent staff introductions and routines may reduce anxiety. In community settings, consider public noise, transport and unfamiliarity.

Case scenario

A caregiver stands behind a client, speaks rapidly and repeats the same instruction louder when the client does not respond.

Correct professional response: Move into the person's field of vision, reduce noise, use a short sentence, check hearing or sensory needs and allow time.

Common mistakes and professional risks

Risks include shouting, testing memory, speaking through relatives, arguing, rushing, using unexplained touch, giving multiple instructions and treating non-response as defiance.

Key practice points

1. Approach from the front and identify yourself.
2. Use short, respectful language and wait.
3. Reduce noise and support hearing and vision.
4. Listen for emotion and purpose, not only facts.
5. Do not use communication to force compliance.

Lesson assessment

1. **MCQ:** The most supportive opening is: A) "You know me" ; B) "Good morning, I am Ada, here to support you" ; C) shouting the person's name; D) asking five questions at once. **Answer: B. Objective tested: 2.**
2. **Scenario:** The person does not answer immediately. **Answer:** Wait, check sensory needs and repeat calmly if needed. **Objective tested: 2–3.**
3. **True/False:** Louder speech always improves understanding. **Answer: False. Objective tested: 2.**
4. **What should the caregiver NOT do?** Grab a person from behind to redirect them. **Answer:** It can frighten and increase distress. **Objective tested: 2–3.**
5. **Application:** Name two environmental changes that support communication. **Answer:** Reduce noise, improve lighting, face the person, remove competing conversations. **Objective tested: 2.**
6. **Scenario:** A client says they want to "go home." **Answer:** Explore the feeling or need and reassure rather than immediately arguing about the address. **Objective tested: 2–**

Lesson 5. Confusion, Disorientation and Reassurance

Why this matters in professional caregiving

A person living with dementia may be unsure of time, place, people or what is happening. Repeated correction can increase fear and conflict. Reassurance does not mean agreeing with every mistaken belief or providing false information. It means acknowledging the person's emotional experience, offering simple orientation and helping them feel safe.

Learning objectives

The learner should be able to respond to confusion; use gentle orientation; distinguish reassurance from arguing or deception; and escalate new or severe disorientation.

Core instructional content

When a person is confused, first ask whether there is immediate danger or an acute change. A person who is suddenly confused, drowsy, feverish, unsteady or unable to perform familiar tasks may require prompt professional review. Do not assume dementia explains a sudden deterioration.

Use orientation aids such as clocks, calendars, signs, familiar objects, consistent routines and clear introductions. Offer one piece of information at a time. "It is morning; you are at home; I am here to help you wash and have breakfast" may be more useful than a long explanation.

When the person believes something that is not accurate, respond to the emotion and purpose. If someone says, "I must collect my children from school," a caregiver might say, "You are worried about them. Let us sit together and check what needs to happen." Do not ridicule, aggressively correct or create elaborate lies. If a safety decision depends on accurate information, involve the responsible professional.

Reassurance may include calm presence, a familiar object, music, prayer if desired, a drink or a quieter space. Ask what usually helps. Avoid using orientation as a test or making the person feel foolish.

Applying This in Practice

In multigenerational homes, relatives may repeatedly correct the person. Explain that calm reassurance is usually more helpful than arguing. In residential settings, consistent signs, staff introductions and predictable routines can reduce disorientation. In community care, identify safe contacts and familiar routes.

Case scenario

A client insists that their deceased spouse is waiting in the next room and becomes distressed when corrected.

Correct professional response: Acknowledge the longing or concern, provide reassurance, avoid a blunt argument and observe whether there are additional signs of acute illness or distress requiring escalation.

Common mistakes and professional risks

Risks include arguing, laughing, testing, saying “you are wrong,” using deception as a routine shortcut, ignoring sudden confusion and overloading the person with information.

Key practice points

1. Check whether confusion is sudden or accompanied by illness signs.
2. Orient gently and simply.
3. Respond to emotion and purpose rather than arguing about every fact.
4. Use reassurance, familiarity and a calm environment.
5. Escalate acute or severe change.

Lesson assessment

1. **Scenario:** A client is suddenly more confused and unsteady. **Answer:** Treat it as a change requiring prompt professional assessment; do not label it routine dementia. **Objective tested: 5.**
2. **MCQ:** Reassurance means: A) arguing until the person agrees; B) acknowledging emotion and supporting safety; C) mocking; D) always lying. **Answer: B. Objective tested: 4–5.**
3. **True/False:** Every inaccurate statement must be corrected immediately. **Answer: False. Objective tested: 4.**
4. **What should the caregiver NOT do?** Use a person’s confusion to frighten them into compliance. **Answer:** This is unsafe and undignified. **Objective tested: 3–4.**
5. **Application:** Give two orientation supports. **Answer:** Clock/calendar, clear signs, familiar objects, consistent introduction or routine. **Objective tested: 4.**
6. **Scenario:** A client repeatedly asks when lunch will be served. **Answer:** Reassure, use a visible routine cue and answer briefly and consistently. **Objective tested: 4.**

Lesson 6. Family and Cultural Communication

Why this matters in professional caregiving

Families often hold essential knowledge about the person's history, language, routines and changes. They may also be tired, worried, grieving or in disagreement with one another. A professional caregiver builds partnership without blaming, gossiping or allowing family pressure to erase the person's voice.

Learning objectives

The learner should be able to invite family knowledge, communicate observations without blame, protect confidentiality, use cultural humility and escalate disagreements appropriately.

Core instructional content

Ask families what the person enjoys, how they communicate discomfort, what routines matter and what usually helps. Treat family information as valuable but not infallible. The person's current preferences and behaviour also matter. Avoid assuming that the oldest relative, loudest relative or financial decision-maker automatically speaks for the client in every matter.

Use objective language: "Today she pushed the cup away three times and said, 'No water.'" Avoid labels such as "She is being stubborn." Describe the effect and invite collaboration: "What has helped when this happens at home?"

Cultural humility means remaining curious, respectful and willing to adapt while still protecting dignity, consent and safety. Language, food, prayer, gender preferences, family presence and household routines can affect care. Ask rather than stereotype. Use authorised interpretation for sensitive matters; do not place a child in the role of interpreter for confidential or complex communication.

Families may disagree about routines, safety, care setting or the meaning of behaviour. The caregiver should not mediate beyond role or take sides. Record the relevant concern and refer to the supervisor or responsible professional. Protect the client from arguments during care.

Applying This in Practice

In Nigerian settings, extended families may share care responsibilities and decisions. Invite appropriate participation while following the care plan, consent and organisational policy. Recognise that language diversity and religious practice may affect communication and comfort.

Case scenario

A daughter says, “Do not offer him choices; he has dementia.” The client reaches for two shirts and looks between them.

Correct professional response: Offer a simple choice and support the client’s participation. Explain respectfully that the team aims to preserve abilities, and seek professional guidance if there is a genuine safety concern.

Common mistakes and professional risks

Risks include blaming families, gossiping, assuming family authority, disclosing information to unauthorised relatives, stereotyping culture and ignoring the person during family meetings.

Key practice points

1. Families are partners, not automatic substitutes for the person.
2. Use observations rather than blame.
3. Ask about culture and preferences; do not stereotype.
4. Protect confidentiality and use authorised communication support.
5. Escalate disputes rather than taking sides.

Lesson assessment

1. **MCQ:** A useful family question is: A) “Why is she difficult?” ; B) “What usually helps when she becomes worried?” ; C) “Who is to blame?” ; D) “Can we ignore her preference?” **Answer: B. Objective tested: 6.**
2. **Scenario:** A family member demands that the client not be offered choices. **Answer:** Continue person-centred support and escalate genuine safety concerns appropriately. **Objective tested: 6.**
3. **True/False:** Cultural humility means accepting every request without considering rights or safety. **Answer: False. Objective tested: 6.**
4. **What should the caregiver NOT do?** Discuss the client’s behaviour with unrelated neighbours. **Answer:** It breaches confidentiality and dignity. **Objective tested: 8.**
5. **Application:** Rewrite “He is stubborn” objectively. **Answer:** “He pushed the plate away and said, ‘No food,’ on three occasions.” **Objective tested: 8.**
6. **Scenario:** Relatives argue during care. **Answer:** Protect the person from distress, pause the task if needed and refer the disagreement through the appropriate route. **Objective tested: 6–8.**

Module 3 — Behaviour as Communication

Lesson 7. Reading Behaviour as Information

Why this matters in professional caregiving

Calling behaviour “challenging” can end the inquiry too early. Behaviour may be the most available way for a person to communicate pain, fear, frustration, boredom, loneliness, urgency, a wish to leave, a need for control or a response to the environment. This does not mean every behaviour has one obvious meaning. It means the caregiver should investigate the context through observation and appropriate escalation rather than punishment or automatic control.

Learning objectives

The learner should be able to describe behaviour neutrally; ask what it may communicate; identify possible contributing factors; and distinguish observation from diagnosis.

Core instructional content

Use descriptive language. “Mr Musa walked quickly between the door and window, called ‘home’ and pushed my hand away” is more useful than “Mr Musa was aggressive.” Description supports pattern recognition and avoids defining the person by one episode.

Consider the ABC pattern: **Antecedent**—what happened before; **Behaviour**—what the person did or communicated; **Consequence**—what happened afterward. Add the person, place, time, health, sensory state and task. The purpose is not to create a simplistic formula but to see relationships and test safer responses.

Possible needs include pain, hunger, thirst, toileting, constipation, fatigue, fear, grief, boredom, loneliness, overstimulation, under-stimulation, loss of control, difficulty with language, hearing or vision, unfamiliar staff, a task that is too complex or a memory linked to the environment. Medication effects, infection or delirium may also contribute and require professional assessment.

Avoid interpreting behaviour as intentional misbehaviour unless there is clear evidence and professional context. Avoid punishment, shouting, ridicule, threats, unnecessary restraint and withholding privileges. A response that stops behaviour temporarily may still increase fear and reduce trust.

Applying This in Practice

Use neutral observation in handover and records. Share patterns with family and the care team. If a behaviour creates immediate danger, seek help and protect everyone without

humiliating or blaming the person.

Case scenario

A client repeatedly pulls at their clothing and calls out shortly after meals. Staff describe the person as “attention-seeking.”

Correct professional response: Observe the timing, check comfort, toileting, pain, temperature and clothing, describe the behaviour objectively and report the pattern for assessment. Do not assume motive.

Common mistakes and professional risks

Risks include labels, punishment, assuming manipulation, ignoring physical causes, recording only the behaviour and not the context, and failing to escalate repeated or dangerous distress.

Key practice points

1. Describe what happened, not who the person supposedly is.
2. Ask what the behaviour may be communicating.
3. Consider physical, emotional, environmental and communication factors.
4. Do not diagnose or assume intent.
5. Document patterns and responses.

Lesson assessment

1. **MCQ:** Which is the best documentation? A) “She was difficult” ; B) “She pushed the cup away and turned her head” ; C) “She wanted attention” ; D) “She was bad.”
Answer: B. Objective tested: 3.
2. **Scenario:** A person calls out repeatedly. **Answer:** Observe timing, context, needs and response; provide calm support and document. **Objective tested: 3–4.**
3. **True/False:** Behaviour always has one obvious meaning. **Answer: False. Objective tested: 3.**
4. **What should the caregiver NOT do?** Punish a person for expressing distress. **Answer:** It can increase fear and harm dignity. **Objective tested: 3–4.**
5. **Application:** Name four possible contributors to agitation. **Answer:** Pain, toileting, fatigue, noise, fear, hunger, infection, sensory loss or communication difficulty.
Objective tested: 4.

6. **Scenario:** Behaviour creates immediate danger. **Answer:** Protect safety, call for authorised help and continue respectful communication. **Objective tested: 5.**

Lesson 8. Repetitive Behaviour, Calling Out and Withdrawal

Why this matters in professional caregiving

Repetition may be a way to seek reassurance, remember a task, manage anxiety, fill time or maintain connection. Withdrawal may reflect depression, fatigue, hearing difficulty, pain, fear or overstimulation. The caregiver should avoid showing irritation and instead identify the person's need and adapt the environment or interaction.

Learning objectives

The learner should be able to respond to repetitive questions and actions; support a withdrawn person; identify possible needs and triggers; and use consistent, non-shaming strategies.

Core instructional content

When a person repeats a question, respond briefly and calmly. Use a written or visual cue if helpful, such as a simple schedule or clock. Avoid saying "I already told you" in an irritated tone. If repetition is linked to a need—food, an appointment, going home, a missing object—address the need where possible.

Repetitive actions such as folding, checking a door, collecting objects or pacing may offer comfort or purpose. Consider whether the action is safe and meaningful before stopping it. Redirect only when there is risk, damage or severe distress. Provide a related activity rather than a random distraction.

Withdrawal requires curiosity, not pressure. Check hearing, vision, pain, mood, fatigue, medication effects and the social environment. Approach gently and offer brief connection. Do not label the person rude or uninterested.

Consistency helps. Agree with the team on language and responses so that the person does not receive contradictory messages. Record what helps and what increases distress.

Applying This in Practice

In home settings, family members may become frustrated with repeated questions. Suggest a visible routine, a calm repeated answer and scheduled reassurance. In residential care, use consistent staff approaches and meaningful tasks. In community care, consider whether public environments are overwhelming.

Case scenario

A client asks every five minutes whether their son is coming. The caregiver says, “Stop asking.” The client becomes tearful.

Correct professional response: Acknowledge the worry, answer briefly, provide a visual cue or plan if appropriate, offer reassurance and consider whether loneliness, uncertainty or a specific unmet need is driving the repetition.

Common mistakes and professional risks

Risks include irritation, humiliation, unnecessary sedation requests, stopping harmless activity, ignoring withdrawal and using distraction without understanding the person’s need.

Key practice points

1. Repetition may communicate anxiety or a need for reassurance.
2. Use consistent, brief responses and visual cues.
3. Do not stop safe, meaningful repetitive activity automatically.
4. Withdrawal deserves observation and possible escalation.
5. Record what helps.

Lesson assessment

1. **Scenario:** A client repeats the same question. **Answer:** Respond calmly, acknowledge worry and use a simple cue or plan. **Objective tested: 7–8.**
2. **MCQ:** Repetitive activity should be stopped: A) always; B) only when unsafe, harmful or seriously distressing, with a safer alternative; C) because it annoys staff; D) by force. **Answer: B. Objective tested: 7.**
3. **True/False:** Withdrawal can indicate pain, low mood, fatigue or communication difficulty. **Answer: True. Objective tested: 8.**
4. **What should the caregiver NOT do?** Shame the person for repeated questions. **Answer:** It can increase distress. **Objective tested: 7.**
5. **Application:** Give two supports for repetitive reassurance-seeking. **Answer:** Short consistent response, visual schedule, familiar object, planned check-in or meaningful activity. **Objective tested: 8.**
6. **Scenario:** A safe repetitive task is meaningful to the client. **Answer:** Allow it where possible and monitor safety. **Objective tested: 7.**

Lesson 9. Observation and Trigger Mapping

Why this matters in professional caregiving

A single episode can be difficult to interpret. A pattern across time, task, environment and response is more useful. Trigger mapping helps the team move from “this always happens” to a more practical question: “What is happening before, during and after, and what response reduces distress?”

Learning objectives

The learner should be able to complete a simple observation record; identify antecedents, behaviour and outcomes; distinguish facts from interpretation; and share patterns for review.

Core instructional content

A useful record includes date and time, location, people present, task or event before the behaviour, the person’s exact words where possible, observable actions, body-language and health cues, environmental factors, the caregiver’s response, the outcome and who was informed.

Look for triggers such as rushed personal care, unfamiliar staff, noise, crowding, hunger, missed toileting, change in routine, poor lighting, a mirror, a television programme, a particular visitor, pain, fatigue or a difficult question. Also look for protective factors: a familiar song, quiet room, preferred caregiver, snack, walk, prayer, rest or a step-by-step explanation.

Do not turn a trigger map into a diagnosis. “Distress occurred after the shower was started” does not prove the shower caused it; it signals a need to review pain, temperature, privacy, fear, task sequence and communication.

Use records in handover and care-plan review. If a pattern persists or becomes more severe, escalate. A documented pattern can help authorised professionals assess pain, infection, medication effects, mood or environmental needs.

Applying This in Practice

Use a simple paper or electronic form approved by the organisation. Avoid storing personal notes on a private phone. Ensure observations are shared confidentially and with people who need them for care.

Case scenario

A client resists care on three mornings but accepts it after lunch. The team has been recording “refused care” only.

Correct professional response: Expand the record to include time, approach, task sequence, sleep, pain, food, staff, environment and response. Consider whether timing, fatigue, fear or rushed mornings contribute and escalate the pattern for review.

Common mistakes and professional risks

Risks include vague labels, missing context, recording only refusal, assuming correlation proves cause, failing to record successful strategies and not sharing repeated patterns.

Key practice points

1. Record antecedent, behaviour, response and outcome.
2. Include health, sensory and environmental context.
3. Separate observation from interpretation.
4. Record what helps as well as what goes wrong.
5. Escalate repeated, worsening or risky patterns.

Lesson assessment

1. **MCQ:** Which belongs in an observation record? A) “He is manipulative” ; B) time, context, observable behaviour, response and outcome; C) staff gossip; D) a diagnosis invented by the caregiver. **Answer: B. Objective tested: 9.**
2. **Scenario:** Resistance occurs only before breakfast. **Answer:** Map timing, sleep, hunger, task sequence, staff approach and environment; report the pattern. **Objective tested: 9.**
3. **True/False:** A trigger map proves causation. **Answer: False.** It identifies patterns for further assessment. **Objective tested: 9.**
4. **What should the caregiver NOT do?** Keep a hidden personal record on a private device. **Answer:** Use approved systems. **Objective tested: 8–9.**
5. **Application:** Name three protective factors to record. **Answer:** Familiar music, quiet space, preferred staff, a walk, food, rest or clear explanation. **Objective tested: 9.**
6. **Scenario:** The behaviour becomes more frequent and risky. **Answer:** Protect safety and escalate with the documented pattern. **Objective tested: 7 and 9.**

Module 4 — Distress, Agitation and Resistance to Care

Lesson 10. Agitation and Distress

Why this matters in professional caregiving

Agitation can include pacing, shouting, repeated movement, pushing away, restlessness, verbal anger or attempts to leave. It is distressing for the person and others, but it should not be treated as a moral failure. NICE recommends structured assessment of distress to explore clinical and environmental causes such as pain, delirium, infection, inappropriate care, boredom, noise and temperature before treatment is initiated ³.

Learning objectives

The learner should be able to recognise distress; use immediate de-escalation; identify possible physical and environmental contributors; and escalate concerns beyond caregiver scope.

Core instructional content

First regulate yourself. Lower your voice, slow your movements, reduce the number of people speaking, give space and avoid blocking the person unless immediate safety requires it. Use the person's name and a short reassuring phrase. Do not demand eye contact, argue or crowd.

Check immediate needs within role: pain cues, toileting, hunger, thirst, temperature, clothing, fatigue, noise, lighting, unfamiliar people and whether the current task is too difficult. Look for signs of illness or acute change and report them. Caregivers must not diagnose infection, delirium or medication effects, but they should treat sudden or severe distress as a reason for prompt review.

Offer a simple choice or a quieter place. If the task is not urgent, pause and return later. If someone is at risk of harming themselves or another person, seek authorised assistance and follow the emergency plan. Do not use restraint, intimidation or medication unless a qualified and authorised process applies; caregivers must not improvise.

After the episode, document what happened before, the person's communication, response and outcome. Review with the team what could be adapted next time.

Applying This in Practice

In busy Nigerian households or facilities, crowding and multiple voices can increase agitation. Ask relatives to reduce stimulation and nominate one calm communicator. Consider heat, generator noise, television, visitors, prayer or meal routines and unfamiliar staff.

Case scenario

During a noisy family visit, a client begins pacing, shouting and pushing away anyone who approaches. A relative tells staff to hold the client down.

Correct professional response: Reduce noise and people, give space, use calm communication, check safety and seek authorised help. Do not restrain without an authorised, trained and necessary procedure.

Common mistakes and professional risks

Risks include shouting, surrounding the person, arguing, restraint for convenience, assuming deliberate aggression, ignoring pain or infection and failing to report repeated distress.

Key practice points

1. Distress is information, not a character judgement.
2. Reduce stimulation and use one calm communicator.
3. Check physical and environmental contributors.
4. Pause non-urgent care and return later if safe.
5. Escalate sudden, severe or unexplained distress.

Lesson assessment

1. **Scenario:** A person becomes agitated in a noisy room. **Answer:** Reduce noise and people, give space, communicate calmly and assess possible needs. **Objective tested:** 5.
2. **MCQ:** Before treatment for distress, the team should explore: A) only staff frustration; B) clinical, environmental, medication, communication and sensory factors; C) punishment; D) restraint first. **Answer: B. Objective tested: 4–5.**
3. **True/False:** Caregivers may improvise physical restraint if the person is upsetting others. **Answer: False. Objective tested: 5.**
4. **What should the caregiver NOT do?** Crowd the person while asking multiple questions. **Answer:** It can increase threat and distress. **Objective tested: 5.**
5. **Application:** Name three immediate de-escalation actions. **Answer:** Lower voice, reduce stimulation, give space, pause task, use simple reassurance, seek help. **Objective tested: 5.**
6. **Scenario:** Distress is sudden and accompanied by fever or unusual sleepiness. **Answer:** Escalate promptly for professional assessment. **Objective tested: 4 and 7.**
7. **Documentation question:** Record antecedent, behaviour, response, outcome and who was informed. **Answer:** Correct. **Objective tested: 9.**

Lesson 11. Resistance to Personal Care

Why this matters in professional caregiving

Bathing, dressing, toileting and continence care involve privacy, touch, exposure and loss of control. Resistance may communicate fear, pain, embarrassment, cold, trauma, misunderstanding, fatigue, modesty, a rushed approach or a preference for a different time or caregiver. Forcing care can cause injury and destroy trust.

Learning objectives

The learner should be able to identify possible reasons for resistance; adapt timing, task sequence and communication; preserve privacy; and escalate when essential care cannot safely be completed.

Core instructional content

Prepare the environment and supplies before beginning. Explain one step at a time. Ask permission before touch. Offer choices about time, clothing, water temperature, who is present and the order of tasks. Use familiar staff or routines where possible. Consider whether a shorter wash, face-and-hands wash or later attempt meets the immediate need safely.

Check for pain, skin problems, cold, fatigue, constipation, urinary urgency, fear of falling, embarrassment, sensory sensitivity and past experiences. A person may resist because the caregiver's approach is too fast or because they do not understand why an unfamiliar person is touching them.

If the person says no, pause. Do not threaten, shame, trick, pull, hold or expose them. If there is immediate risk from missed care or a serious health concern, report to the responsible professional. Do not decide that the person is incapable simply because they refuse.

A two-person approach may be appropriate in some care plans for safety, but it should never be used as a way to overpower a person or humiliate them. Follow authorisation, privacy and least-restrictive principles.

Applying This in Practice

In home care, family pressure may be high because routines are tied to household schedules. Explain that safe care requires cooperation and dignity. In residential settings, review staffing, gender preferences and timing. In healthcare-support settings, follow the care plan and clinical escalation route.

Case scenario

A client refuses a shower after a new caregiver enters without knocking. The client pulls the blanket up and says, “Leave me.”

Correct professional response: Stop, apologise for the privacy breach, give space, explain who you are, offer a later time or alternative and report if the care need remains unmet.

Common mistakes and professional risks

Risks include forcing, bribing, exposing, rushing, ignoring gender or modesty preferences, assuming refusal equals confusion and recording only “non-compliant.”

Key practice points

1. Resistance is often a signal about the care approach or the person’s needs.
2. Ask permission and protect privacy.
3. Adapt timing, sequence, staff, environment and task size.
4. Never use force or humiliation for convenience.
5. Escalate when necessary care cannot safely be completed.

Lesson assessment

1. **Scenario:** The person refuses bathing. **Answer:** Pause, explore possible reasons, offer choices or a later attempt and protect dignity. **Objective tested: 5–6.**
2. **MCQ:** Which is least appropriate? A) offer a later time; B) use a familiar caregiver; C) shame the person; D) explain one step at a time. **Answer: C. Objective tested: 5.**
3. **True/False:** “Non-compliant” is sufficient documentation. **Answer: False.** Record observable behaviour and context. **Objective tested: 9.**
4. **What should the caregiver NOT do?** Pull a person into the bathroom because the schedule is tight. **Answer:** It risks injury and violates dignity. **Objective tested: 5–6.**
5. **Application:** Name four possible contributors to resistance. **Answer:** Pain, cold, fear, embarrassment, unfamiliar caregiver, fatigue, sensory issue or loss of control. **Objective tested: 4–5.**
6. **Scenario:** Essential continence care cannot be completed safely. **Answer:** Protect privacy, seek help and escalate according to the care plan. **Objective tested: 7 and 9.**
7. **Documentation question:** What should follow a refusal? **Answer:** Record explanation, person’s words/actions, alternatives, outcome and person informed. **Objective tested: 9.**

Lesson 12. De-escalation and Non-Pharmacological Support

Why this matters in professional caregiving

Most everyday distress can be supported through communication, environmental adjustment, meaningful activity, comfort and relationship. The aim is not to “win” or silence the person. It is to reduce fear, meet a need, preserve dignity and protect safety. Caregivers must not recommend or administer medication for behaviour outside authorisation.

Learning objectives

The learner should be able to use person-centred de-escalation; select non-pharmacological supports; evaluate whether an approach helps; and escalate when distress persists or creates risk.

Core instructional content

Non-pharmacological support may include a quiet environment, familiar music, accompanied walking, sensory comfort, a drink or snack where appropriate, toileting, rest, meaningful activity, a familiar person, prayer if desired, reassurance, a change in lighting or temperature and simplifying the task. The appropriate response depends on the person and the possible trigger.

De-escalation should be proportionate. Introduce yourself, speak slowly, validate emotion, reduce demands, offer one choice and allow time. Avoid standing over the person, arguing about facts, rapid touch or a large audience. If an approach increases distress, stop and try another safe option.

The caregiver should evaluate the outcome. Did the person become calmer? Did the possible need get addressed? Was the person's dignity preserved? Is there a repeating pattern? Record effective strategies in the care plan through the authorised process.

If distress persists, worsens or is accompanied by injury, sudden change, hallucinations, fever, severe pain, inability to drink or other concerning signs, escalate. Do not treat non-pharmacological care as a reason to delay urgent assessment.

Applying This in Practice

Use culturally and personally meaningful options, not generic activities. A familiar song, a quiet prayer, a household task or an accompanied walk may be more effective than an unfamiliar activity. Ask the person and family what has previously helped.

Case scenario

A client is pacing and repeatedly opening the gate. The caregiver blocks the gate and argues that the client must stay inside.

Correct professional response: Assess immediate safety, approach calmly, explore whether the person wants to go somewhere or is searching for someone, offer an accompanied walk or meaningful alternative if safe, and escalate if the risk of getting lost is significant. Do not rely only on confrontation.

Common mistakes and professional risks

Risks include using one intervention for everyone, treating medication as the first answer, distracting without listening, blocking movement unnecessarily, ignoring acute symptoms and failing to evaluate outcomes.

Key practice points

1. Calm, personalised, non-pharmacological support is usually the first practical response.
2. Match the intervention to the person and possible need.
3. Stop an approach that increases distress.
4. Do not recommend or administer behaviour medication outside role.
5. Escalate persistent, severe or medically concerning distress.

Lesson assessment

1. **MCQ:** A suitable first response to distress may include: A) quiet, reassurance and meaningful activity; B) threats; C) automatic sedation; D) public correction. **Answer: A. Objective tested: 5.**
2. **Scenario:** Walking reduces distress and is safe with supervision. **Answer:** Include it as a personalised support and document the effective approach. **Objective tested: 6.**
3. **True/False:** Non-pharmacological support means clinical escalation is never needed. **Answer: False. Objective tested: 7.**
4. **What should the caregiver NOT do?** Recommend a sedative because the person is noisy. **Answer:** Medication decisions require authorised clinicians. **Objective tested: 10.**
5. **Application:** Name three de-escalation strategies. **Answer:** Validation, reduced noise, space, simple choice, familiar music, accompanied walk or comfort. **Objective tested: 5–6.**
6. **Scenario:** A strategy increases agitation. **Answer:** Stop, return to safety, reassess the approach and seek help if needed. **Objective tested: 5.**
7. **Documentation question:** Record whether the intervention reduced distress and any next action. **Answer:** Correct. **Objective tested: 9.**

Module 5 — Safety, Wandering and Sudden Change

Lesson 13. Wandering, Getting Lost and Environmental Safety

Why this matters in professional caregiving

Walking and exploring may be meaningful, purposeful or soothing. The term “wandering” can make ordinary movement sound like a problem, yet getting lost, entering unsafe areas, crossing roads, falling or leaving without support can create serious risk. The professional aim is to preserve safe movement and independence while reducing foreseeable danger.

Learning objectives

The learner should be able to distinguish movement from immediate risk; identify possible reasons for leaving; reduce environmental hazards; and escalate repeated or high-risk episodes.

Core instructional content

Ask what the person may be trying to do. They may be looking for a person, workplace, toilet, home, food, activity, familiar route or relief from boredom. They may be responding to a memory, routine or internal sense of time. Do not assume that movement has no purpose.

Safety planning may include identification information, agreed contacts, safe walking routes, supervision, lighting, clear signage, secure but non-institutional environments, removing trip hazards, door alarms where authorised, accessible toilets, meaningful activity and regular movement. Avoid locking a person in, hiding shoes or using surveillance secretly unless an authorised, proportionate and reviewed plan applies.

When a person is missing or at immediate risk, follow the organisation’s emergency procedure. Notify the supervisor and relevant emergency services as required. Do not delay while trying to solve the situation privately. Record the circumstances and review the care plan.

Safety does not justify removing all freedom. A person may need accompanied walking rather than confinement. Discuss risk with the responsible professional and family, considering the person’s wishes and the least restrictive safe option.

Applying This in Practice

In Nigerian homes, consider compound gates, traffic, open drainage, heat, rain, generator areas, cooking fires, water bodies and community familiarity. In residential care, use

individual risk planning rather than blanket locked doors. In community settings, identify safe routes and emergency contacts.

Case scenario

A client repeatedly walks to the gate at 17:00 and says they need to collect their children. Staff want to lock the client in their room.

Correct professional response: Explore the meaning and pattern, offer a safe accompanied walk or meaningful routine, address environmental risk and escalate for an individualised plan. Do not use unnecessary confinement.

Common mistakes and professional risks

Risks include locking doors for convenience, hiding belongings, treating movement as misconduct, failing to check for purpose, ignoring traffic and heat risks and delaying an emergency response when someone is missing.

Key practice points

1. Movement may be meaningful; assess risk rather than stopping it automatically.
2. Identify patterns, purpose, routes and hazards.
3. Use the least restrictive safe plan.
4. Escalate missing-person or immediate-danger situations promptly.
5. Review plans after changes or incidents.

Lesson assessment

1. **Scenario:** A client walks repeatedly to the gate. **Answer:** Explore purpose, assess hazards, offer safe movement and seek an individualised plan. **Objective tested: 7.**
2. **MCQ:** A proportionate response is: A) lock the person in a room; B) accompanied walking and environmental safety planning; C) remove shoes; D) threaten punishment. **Answer: B. Objective tested: 6–7.**
3. **True/False:** All wandering should be stopped. **Answer: False. Objective tested: 6.**
4. **What should the caregiver NOT do?** Hide a client's clothing to prevent leaving. **Answer:** It is undignified and may be restrictive. **Objective tested: 6.**
5. **Application:** Name three environmental risks to review. **Answer:** Traffic, heat, open drains, water, fire, poor lighting, trip hazards or unsafe gates. **Objective tested: 7.**
6. **Scenario:** The person is missing from the service. **Answer:** Follow the emergency pathway immediately and document actions. **Objective tested: 7 and 9.**

7. **Documentation question:** Record time, last location, clothing, route, possible purpose and notifications. **Answer:** Correct. **Objective tested: 9.**

Lesson 14. Sundowning, Falls and Time-of-Day Changes

Why this matters in professional caregiving

Some people experience more confusion, restlessness or distress later in the day. This pattern is often called sundowning, but not every evening change has the same cause. Fatigue, poor lighting, noise, hunger, pain, medication timing, disrupted sleep, overstimulation or unmet needs may contribute. Falls and night-time risks also require proactive, person-centred planning.

Learning objectives

The learner should be able to identify time-of-day patterns; adapt routine and environment; reduce fall risks; and escalate repeated or new changes.

Core instructional content

Record when symptoms start, what happened during the day, sleep, food, fluids, activity, visitors, lighting, medication timing as documented by authorised staff and the person's response to support. Avoid claiming that sunset itself explains every change.

Helpful adaptations may include a predictable evening routine, reduced noise, adequate lighting without glare, toileting, comfortable temperature, a snack or drink where appropriate, calm activity, familiar music, rest and safe movement. Avoid exhausting the person or keeping them awake as a routine strategy without professional advice.

Falls prevention includes clearing hazards, appropriate footwear, lighting, accessible mobility aids, safe seating, supervision according to plan and reporting changes in balance or gait. Do not move or lift a person after a serious fall unless trained and the situation requires immediate safety; seek authorised help.

A new pattern of night-time confusion, repeated falls, increased sleepiness, pain, weakness or difficulty walking requires escalation. The caregiver should not adjust medication, restrain the person or use bed rails as an automatic solution.

Applying This in Practice

Consider heat, power interruptions, dim compounds, evening prayer, television noise, visitors, cooking activity and family routines. Work with the household to reduce competing stimulation and maintain a familiar, safe pattern.

Case scenario

A client becomes restless at dusk and has fallen twice in the last week. A relative says it is normal sundowning.

Correct professional response: Document the timing and falls, assess immediate safety, report promptly for professional review and adapt the environment. Do not dismiss the falls as routine.

Common mistakes and professional risks

Risks include dismissing falls, changing medication without authority, using restraint, poor lighting, sleep deprivation, clutter and failing to track patterns.

Key practice points

1. Time-of-day patterns still require context and observation.
2. Use predictable, calm routines and adequate lighting.
3. Treat falls and new mobility changes seriously.
4. Do not alter medication or use restraint outside authority.
5. Escalate repeated or new night-time risks.

Lesson assessment

1. **Scenario:** Evening restlessness accompanies repeated falls. **Answer:** Report the falls and change promptly, protect safety and review the environment. **Objective tested: 7.**
2. **MCQ:** A useful evening support is: A) adequate lighting and predictable routine; B) locked room; C) shouting; D) withholding fluids. **Answer: A. Objective tested: 7.**
3. **True/False:** “Sundowning” means no further assessment is necessary. **Answer: False. Objective tested: 7.**
4. **What should the caregiver NOT do?** Change medication timing independently. **Answer:** Medication changes require authorised professionals. **Objective tested: 10.**
5. **Application:** Name four factors to record around evening distress. **Answer:** Sleep, food/fluid, activity, visitors, lighting, noise, pain, toileting or timing. **Objective tested: 9.**
6. **Scenario:** A client falls. **Answer:** Follow the incident and clinical pathway; do not move them unnecessarily or conceal the fall. **Objective tested: 7 and 9.**
7. **Documentation question:** Record time, circumstances, injury signs, action and person informed. **Answer:** Correct. **Objective tested: 9.**

Lesson 15. Sudden Changes, Delirium Concerns and Escalation

Why this matters in professional caregiving

A sudden change in attention, alertness, communication, mobility, behaviour or continence may be a sign of illness, pain, infection, dehydration, medication effect, injury or delirium rather than gradual dementia progression. Delirium is a clinical condition and caregivers do not diagnose it, but prompt recognition and escalation can be important.

Learning objectives

The learner should be able to identify concerning change from baseline; distinguish gradual pattern from sudden deterioration; take immediate safety steps; and escalate without diagnosis or delay.

Core instructional content

Red flags include sudden confusion, unusual drowsiness, rapid change in speech, new hallucinations, fever, severe pain, weakness, facial drooping, new falls, inability to walk, reduced intake, breathing difficulty, seizure, head injury, severe agitation or a rapid change in continence. Follow emergency procedures for life-threatening signs.

Compare with the person's usual baseline. Ask colleagues or family about timing, but do not wait for everyone to agree if the change is serious. Record what was observed, when it began if known, associated signs, recent falls or illness, food and fluid intake and who was contacted.

Do not give extra medication, change doses, restrain, diagnose delirium or reassure the family that it is "just dementia." Keep the person safe, calm and supervised as appropriate while seeking professional help. Use the communication channel specified by the service and confirm that the concern was received.

A gradual change can also matter. Increased resistance, withdrawal, poor appetite or repeated falls should be discussed with the care team rather than normalised.

Applying This in Practice

In home care, know the emergency contact and clinical escalation route. In residential care, use handover and incident systems. In community settings, do not leave a suddenly unwell person alone or rely on an informal neighbour to decide what to do.

Case scenario

A client with established dementia is suddenly unable to stand, is sweating and is speaking incoherently. A family member says the client often becomes confused.

Correct professional response: Treat it as an urgent change, protect the person, seek immediate authorised medical assessment and record the signs and time. Do not attribute

it to dementia or wait until the next routine visit.

Common mistakes and professional risks

Risks include normalising acute change, delaying help, giving unauthorised medication, diagnosing, relying on family reassurance and failing to record onset or associated signs.

Key practice points

1. Sudden change is not automatically dementia progression.
2. Recognise red flags and escalate urgently.
3. Keep the person safe while awaiting help.
4. Do not diagnose or change treatment.
5. Record onset, signs, actions and notifications.

Lesson assessment

1. **MCQ:** Sudden confusion with weakness requires: A) routine observation only; B) prompt professional or emergency escalation; C) punishment; D) extra medication from the caregiver. **Answer: B. Objective tested: 7 and 10.**
2. **Scenario:** A family member says “that is normal.” **Answer:** Compare baseline, document the acute change and escalate based on the signs. **Objective tested: 7.**
3. **True/False:** Caregivers should diagnose delirium if the signs look familiar. **Answer: False. Objective tested: 10.**
4. **What should the caregiver NOT do?** Wait until the next day when severe symptoms are present. **Answer:** Use the urgent pathway. **Objective tested: 7.**
5. **Application:** Name four red flags. **Answer:** Sudden drowsiness, weakness, fever, new fall, severe pain, breathing difficulty, rapid speech change or inability to walk. **Objective tested: 7.**
6. **Scenario:** The person becomes suddenly agitated after a fall. **Answer:** Protect safety, seek assessment and report the fall and change. **Objective tested: 7 and 9.**
7. **Documentation question:** What timing matters? **Answer:** When the change was first noticed, associated signs, action and person informed. **Objective tested: 9.**

Module 6 — Partnership, Documentation and Professional Response

Lesson 16. Family Partnership and Care Planning

Why this matters in professional caregiving

Good dementia care is coordinated. Families and caregivers can notice different patterns and offer different forms of knowledge. Partnership helps the team understand the person, but it must not become blame, secrecy or an excuse to override the person's choices.

Learning objectives

The learner should be able to share observations respectfully, invite family expertise, contribute to care-plan review and raise concerns without blame.

Core instructional content

Use structured communication: describe the situation, explain the observed pattern, state what was tried, report the outcome and ask for relevant information. “He became distressed during shaving at 08:00, pushed the razor away and calmed after we paused. Has this happened before?” is more useful than “He refuses everything.”

Invite family knowledge about routines, preferred language, comfort, meaningful activities, past trauma, mobility, food and signs of pain. Verify important information through the care plan and responsible professionals. Do not ask family to perform tasks outside their role or use them to conceal incidents.

Care plans should include communication preferences, triggers, successful responses, safety risks, meaningful activities, personal-care preferences, escalation instructions and relevant contacts. Caregivers should report when the plan no longer reflects the person's needs. They should not alter clinical instructions independently.

Family distress is real. Listen respectfully, acknowledge concern and refer complaints or disagreements through the appropriate process. Do not argue at the bedside or discuss another family's client.

Applying This in Practice

In Nigeria, family networks may be central to care coordination. Clarify who is authorised to receive information, how updates are shared and who should be contacted in an urgent change. Consider language, travel, finances and availability without treating one relative as automatically responsible for all decisions.

Case scenario

A family says the client has “always been aggressive,” but the records show that distress occurs only during a particular personal-care task.

Correct professional response: Share the pattern without blame, ask what the task means to the client, review communication and environment, and request care-plan review rather

than accepting a global label.

Common mistakes and professional risks

Risks include blaming families, oversharing, ignoring family knowledge, discussing disagreements in front of the person, changing a plan informally and failing to update successful strategies.

Key practice points

1. Use observations and patterns, not labels.
2. Invite family knowledge while keeping the person central.
3. Update the care plan through authorised processes.
4. Refer complaints and conflicts appropriately.
5. Clarify contacts and information-sharing arrangements.

Lesson assessment

1. **MCQ:** The best family update is: A) “He is impossible” ; B) a factual pattern, response and question; C) gossip; D) an accusation. **Answer: B. Objective tested: 8.**
2. **Scenario:** Family knowledge conflicts with a current client preference. **Answer:** Respect the current preference where safe and discuss the difference through the care team. **Objective tested: 6 and 8.**
3. **True/False:** Families are valuable sources of information but not automatic substitutes for the person’s voice. **Answer: True. Objective tested: 8.**
4. **What should the caregiver NOT do?** Change a clinical care plan independently. **Answer:** Raise the need for authorised review. **Objective tested: 9–10.**
5. **Application:** Give three care-plan items relevant to dementia behaviour support. **Answer:** Triggers, communication preferences, successful responses, meaningful activities, safety plan or escalation route. **Objective tested: 8–9.**
6. **Scenario:** A relative complains angrily during care. **Answer:** Protect the person, remain calm and use the complaints or supervisory route. **Objective tested: 8.**

Lesson 17. Documentation, Handover and Reporting

Why this matters in professional caregiving

Documentation turns an episode into shared professional knowledge. A clear record helps the next caregiver understand what happened, what may have contributed, what response

worked and what needs follow-up. A vague or judgemental record can cause repeated distress, inappropriate restriction or missed clinical assessment.

Learning objectives

The learner should be able to document behaviour and response objectively; provide an effective handover; record safety concerns; and escalate changes promptly.

Core instructional content

Record date, time, place, people present, task or event before the change, the person's exact words where possible, observable behaviour, health and sensory cues, possible environmental factors, intervention, response, outcome, person informed and follow-up required.

Separate observation from hypothesis. "She held her abdomen, grimaced and declined lunch" is observation. "She was being difficult" is a judgement. "Pain may be contributing; nurse informed" is an appropriately framed concern when escalated.

Handover should be concise and actionable. State what changed, what is currently safe, what helped, what did not help and what must happen next. Do not hand over only a label such as "agitated."

Report sudden, severe, repeated or risky changes through the authorised route. Immediate danger or medical emergency requires immediate communication, not only a later form. If a supervisor is involved in the concern, use the alternative pathway.

Protect records and confidentiality. Use approved systems, do not store client details in personal messaging apps, and do not discuss cases in public or online groups.

Applying This in Practice

Use NIC or employer-approved observation tools. In home care, ensure the family or next worker knows the agreed action without receiving unnecessary confidential details. In residential care, connect documentation to shift handover. In community care, secure notes during transport.

Case scenario

A record says, "Aggressive all afternoon." The next caregiver approaches quickly and the client becomes distressed again.

Correct professional response: Replace vague labels through an objective addendum or correction process, describe timing, trigger, behaviour, response and outcome, and communicate the effective approach.

Common mistakes and professional risks

Risks include judgemental labels, backdating, deleting records, recording only failure, failing to document successful support, informal gossip and delayed escalation.

Key practice points

1. Describe facts, words, patterns and outcomes.
2. Record what happened before and what helped afterward.
3. Use handover to support continuity.
4. Escalate urgent change directly and promptly.
5. Protect records and confidentiality.

Lesson assessment

1. **MCQ:** Which is objective? A) “He was naughty” ; B) “He shouted ‘go away’ and pushed the towel” ; C) “She wanted attention” ; D) “He is aggressive.” **Answer: B. Objective tested: 9.**
2. **Scenario:** A sudden change is recorded only in a form. **Answer:** Use direct urgent escalation as well as documentation. **Objective tested: 7 and 9.**
3. **True/False:** A successful calming strategy should also be recorded. **Answer: True. Objective tested: 9.**
4. **What should the caregiver NOT do?** Use a personal WhatsApp group for client handover. **Answer:** Use approved confidential channels. **Objective tested: 8–9.**
5. **Application:** Name five record elements. **Answer:** Time, context, exact words, behaviour, possible factors, response, outcome, person informed or follow-up. **Objective tested: 9.**
6. **Scenario:** The usual supervisor is implicated. **Answer:** Use the alternative reporting route and document the escalation. **Objective tested: 9–10.**
7. **Handover question:** What does the next caregiver need to know? **Answer:** Current risk, what happened, what helped, what to avoid and the next action. **Objective tested: 9.**

Lesson 18. Integrated Dementia-Care Judgement

Why this matters in professional caregiving

Real situations combine communication, distress, family pressure, safety, dignity and documentation. A caregiver may need to respond to resistance while noticing a sudden change, preserve movement while preventing a person getting lost, or communicate with a

family that wants control. Professional judgement is the ability to remain calm, curious, person-centred and within scope.

Learning objectives

The learner should be able to apply P-O-C-T-A-P-D-E; prioritise dignity and safety; adapt communication; document and escalate; and identify when caregiver action must stop and professional assessment is needed.

Core instructional content

Begin with the person, not the label. Pause and observe. What is the person saying, doing, avoiding or seeking? What changed? What happened immediately before? What could be contributing? Is there a sudden physical or safety concern?

Use the least intrusive approach that may work. Reduce noise, offer space, simplify language, preserve choice and adapt the task. Protect privacy and avoid arguing. If there is immediate danger, seek help while remaining respectful.

Do not diagnose, restrain, medicate, investigate or change care plans outside authority. Report concerning changes and provide objective information. Communicate with family as partners but do not disclose beyond authorisation or allow family pressure to override the person's rights.

Document the episode and evaluate. What helped? What needs review? Does the pattern require a care-plan change, clinical assessment, environmental adaptation, family discussion or safeguarding response?

Applying This in Practice

Use the framework in homes, residential facilities, community visits and hospital-support environments. It should become a shared language for supervision and handover.

Case scenario

A client refuses dinner, repeatedly tries to leave, appears more confused than usual and becomes distressed when a relative insists on feeding them. The relative asks the caregiver to hold the client down and says not to report the change.

Correct professional response: Pause and observe; assess immediate safety; recognise possible unmet need or acute change; reduce pressure and use calm communication; do not force, restrain or conceal; seek urgent authorised professional review because the change is unusual; document the person's words, actions, context, responses and notifications; and maintain confidentiality.

Common mistakes and professional risks

Risks include choosing routine over safety, treating all behaviour as dementia, accepting family pressure, using force, failing to document, delaying escalation and forgetting the person's emotional experience.

Key practice points

1. Start with personhood and observation.
2. Look for triggers without diagnosing.
3. Adapt communication and environment before escalating demands.
4. Protect dignity and safety together.
5. Document and escalate sudden, severe, repeated or risky changes.

Lesson assessment

1. **Integrated scenario:** A person resists care and appears suddenly more confused.
Answer: Pause, protect safety, consider acute causes, adapt care and escalate promptly. **Objective tested: 4–7.**
2. **MCQ:** The correct sequence begins with: A) restraint; B) pause and observe; C) correction; D) medication advice. **Answer: B. Objective tested: 10.**
3. **True/False:** Family pressure can justify hiding a sudden change. **Answer: False. Objective tested: 8–10.**
4. **What should the caregiver NOT do?** Make a diagnosis from one episode. **Answer:** Observe and report. **Objective tested: 1 and 10.**
5. **Application:** List the P-O-C-T-A-P-D-E steps. **Answer:** Pause, Observe, Consider the person, Trace triggers, Approach calmly, Personalise/adapt, Dignity and safety, Enter/escalate/evaluate. **Objective tested: 10.**
6. **Scenario:** The first calming strategy fails. **Answer:** Stop if it increases distress, reassess, seek help and document. **Objective tested: 5 and 9.**
7. **Documentation question:** What makes the record useful? **Answer:** Objective context, exact words, response, outcome, risk and escalation. **Objective tested: 9.**

13. Lesson assessments

The lesson assessments are embedded after each lesson. Each should provide immediate feedback explaining why the correct response preserves personhood, dignity and safety, and why the distractors are unsafe or outside caregiver scope. NIC may randomise equivalent items while retaining the same learning objectives.

14. Module assessments

Each module assessment should contain 6–8 application questions and a recommended threshold of 80%, with one retry after review.

Module 1 assessment

1. **Scenario:** A client's ability changes suddenly. **Answer:** Observe, document and escalate; do not say it is simply dementia. **LO:** 1, 7.
2. **MCQ:** Personhood information includes strengths and preferences. **Answer:** Correct. **LO:** 1–2.
3. **Scenario:** A caregiver takes over dressing despite the person's ability to participate. **Answer:** Offer time, prompts and choices. **LO:** 3, 6.
4. **True/False:** Dementia removes autonomy. **Answer:** False. **LO:** 1, 3.
5. **Scenario:** A meaningful activity is refused. **Answer:** Respect refusal and offer another option later. **LO:** 6.
6. **What should not happen?** Infantilising language or unnecessary restriction. **LO:** 3, 6.

Module 2 assessment

1. **Scenario:** The person does not respond to rapid questions. **Answer:** Reduce pace, check sensory needs and wait. **LO:** 2.
2. **MCQ:** Reassurance should acknowledge emotion rather than argue. **Answer:** Correct. **LO:** 2, 5.
3. **Scenario:** A family member speaks over the client. **Answer:** Address the client directly and use authorised support. **LO:** 8.
4. **True/False:** Cultural respect means every request overrides safety. **Answer:** False. **LO:** 8.
5. **Scenario:** The client wants to go home. **Answer:** Explore emotion and purpose, reassure and assess safety. **LO:** 2, 5.
6. **Documentation:** Replace “difficult” with observable facts. **LO:** 8–9.

Module 3 assessment

1. **Scenario:** Repetitive questioning increases after a visitor leaves. **Answer:** Consider anxiety, loneliness and reassurance needs. **LO:** 3–4.
2. **MCQ:** Trigger mapping includes what happened before, behaviour, response and outcome. **Answer:** Correct. **LO:** 9.

3. **Scenario:** A safe repetitive activity is meaningful. **Answer:** Allow it with appropriate safety. **LO:** 3, 6.
4. **True/False:** A behaviour label is a sufficient record. **Answer:** False. **LO:** 9.
5. **Scenario:** Withdrawal is new. **Answer:** Observe, check possible contributors and escalate if concerning. **LO:** 4, 7.
6. **What should not happen?** Punishment or assuming intent. **LO:** 3–5.

Module 4 assessment

1. **Scenario:** Agitation occurs in a crowded room. **Answer:** Reduce stimulation, give space, use one calm communicator and assess needs. **LO:** 5.
2. **MCQ:** Resistance to bathing may reflect pain, fear, cold or loss of control. **Answer:** Correct. **LO:** 4–5.
3. **Scenario:** The person refuses personal care. **Answer:** Pause, offer choices, adapt timing and escalate unmet essential care. **LO:** 5–7.
4. **True/False:** Restraint is an acceptable shortcut. **Answer:** False. **LO:** 5.
5. **Scenario:** Distress is accompanied by fever. **Answer:** Escalate promptly. **LO:** 7.
6. **What should not happen?** Unauthorised medication advice or administration. **LO:** 10.
7. **Documentation:** Record trigger, response, outcome and notification. **LO:** 9.

Module 5 assessment

1. **Scenario:** A client walks toward a gate each evening. **Answer:** Explore purpose, assess risk and create an individualised safe movement plan. **LO:** 7.
2. **MCQ:** Sudden confusion requires prompt review. **Answer:** Correct. **LO:** 7.
3. **Scenario:** Repeated falls occur at dusk. **Answer:** Report and review safety; do not dismiss as sundowning. **LO:** 7.
4. **True/False:** Locking a person in a room is always appropriate for wandering. **Answer:** False. **LO:** 6–7.
5. **Scenario:** A person is missing. **Answer:** Use the emergency pathway immediately. **LO:** 7, 9.
6. **What should not happen?** Independent medication changes or diagnosis. **LO:** 10.
7. **Documentation:** Record time, location, signs, actions and notifications. **LO:** 9.

Module 6 assessment

1. **Scenario:** Family describes the client as aggressive; records show a task-specific pattern. **Answer:** Share objective observations and review the task. **LO:** 8.
2. **MCQ:** Effective documentation separates observation from interpretation. **Answer:** Correct. **LO:** 9.
3. **Scenario:** Supervisor is implicated in a concern. **Answer:** Use the alternative reporting route. **LO:** 9–10.
4. **True/False:** Online completion alone proves behavioural competence. **Answer:** False. **LO:** 9–10.
5. **Integrated scenario:** Client refuses food, is newly confused and tries to leave. **Answer:** Protect safety, assess acute change, adapt calmly, escalate and document. **LO:** 4–10.
6. **What should not happen?** Hiding change to avoid family conflict. **LO:** 7–9.
7. **Handover:** State what changed, what helped, current risk and next action. **LO:** 8–9.

15. Final assessment blueprint

The final assessment contains **35 questions**. The recommended pass mark is **80% (28/35)**. At least **25 questions** are **scenario/application based**.

Content area	Questions	Number	Application target
Dementia variation, personhood and dignity	1–6	6	4
Communication, confusion and family partnership	7–12	6	5
Behaviour as communication and trigger mapping	13–18	6	5
Distress, agitation and resistance to care	19–24	6	5
Wandering, falls and sudden change	25–30	6	5
Documentation, escalation and integrated judgement	31–35	5	5

16. Final assessment questions

1. **Scenario:** A colleague says every person with dementia eventually becomes aggressive. What is the best response?

A) Agree. B) Recognise individual variation and look for context, needs and triggers. C) Avoid the person. D) Use restraint early.

Answer: B. Dementia is not a single experience and behaviour should not be stereotyped. **LO:** 1.

2. **Scenario:** A client can choose clothes and dress with prompts. What should the caregiver do?

A) Take over. B) Offer choices, prompts and time. C) Ask a relative to decide every day. D) Remove the clothes.

Answer: B. LO: 2, 6.

3. **MCQ:** Person-centred care means: A) focusing only on diagnosis; B) seeing the person's identity, preferences, strengths and needs; C) controlling behaviour; D) treating everyone identically. **Answer: B. LO:** 1–3.

4. **True/False:** Dementia always removes the person's ability to make choices. **Answer: False. LO:** 3.

5. **Scenario:** A familiar activity calms the client. What should happen? **Answer:** Include it in the authorised care information as a meaningful support. **LO:** 2, 6.

6. **What should the caregiver NOT do?** Use childish labels such as “naughty” or “baby.” **Answer:** Such language undermines dignity. **LO:** 3.

7. **Scenario:** A client does not answer a rapid question. What should the caregiver do?

A) Shout. B) Wait, check sensory needs and simplify. C) Assume refusal. D) Ask five more questions.

Answer: B. LO: 2.

8. **Scenario:** A client says, “I want to go home,” despite being at home. What is a helpful first response?

A) Argue. B) Explore the feeling or purpose and reassure. C) Laugh. D) Threaten to call family.

Answer: B. LO: 2, 5.

9. **MCQ:** Reassurance is: A) agreeing with every inaccurate fact; B) acknowledging emotion and supporting safety; C) correcting repeatedly; D) using deception as routine. **Answer: B. LO: 5.**
10. **Scenario:** A family member speaks over the client. What should the caregiver do?
A) Ignore the client. B) Address the client directly and use authorised communication support. C) Share all information with the family. D) End care.
Answer: B. LO: 8.
11. **True/False:** Cultural respect requires accepting a request even when it creates unsafe or undignified care. **Answer: False. LO: 8.**
12. **Scenario:** New disorientation appears during a community visit. What should the caregiver do?
A) Assume it is normal. B) Protect safety, document and escalate the change. C) Leave the person alone. D) Diagnose delirium.
Answer: B. LO: 5, 7.
13. **MCQ:** Behaviour as communication means: A) every behaviour has one clear cause; B) behaviour may signal a need, feeling, discomfort or response to context; C) behaviour should be punished; D) behaviour is always intentional. **Answer: B. LO: 3.**
14. **Scenario:** A client pulls at clothing after meals. What should the caregiver do?
A) Label attention-seeking. B) Observe timing, comfort, toileting, pain and environment, then report the pattern. C) Restrain. D) Ignore.
Answer: B. LO: 3–4.
15. **Scenario:** A safe repetitive task is meaningful. What is appropriate?
A) Stop it automatically. B) Allow it with safety monitoring. C) Shame the person. D) Medicate.
Answer: B. LO: 3, 6.
16. **True/False:** “Aggressive” is sufficient documentation. **Answer: False.** Record observable behaviour and context. **LO: 9.**
17. **Scenario:** Repetition increases after a visitor leaves. What might the caregiver consider?
A) The person is bad. B) Anxiety, loneliness, uncertainty or need for reassurance. C) Punishment. D) Ignoring all questions.
Answer: B. LO: 3–4.
18. **Application:** What belongs in a trigger map? **Answer:** What happened before, observable behaviour, response, outcome, time, place and relevant health or environmental context. **LO: 9.**

19. **Scenario:** A person becomes agitated in a crowded room. What should happen first?
A) Crowd closer. B) Reduce stimulation, give space and use one calm communicator. C) Shout. D) Restrain.
Answer: B. LO: 5.
20. **Scenario:** A person resists bathing after a new caregiver enters without knocking. What is the best response?
A) Force care. B) Stop, apologise, restore privacy and offer a later or adapted approach. C) Call the person difficult. D) Expose the person.
Answer: B. LO: 5–6.
21. **MCQ:** Possible contributors to resistance include: A) pain, fear, cold, embarrassment and loss of control; B) only stubbornness; C) bad character; D) lack of punishment.
Answer: A. LO: 4–5.
22. **True/False:** Caregivers may give extra medication to stop shouting. **Answer: False. LO: 10.**
23. **Scenario:** A relative tells staff to hold a distressed client down. What should the caregiver do?
A) Restrain for convenience. B) Protect safety, de-escalate and seek authorised help. C) Agree automatically. D) Leave.
Answer: B. LO: 5, 7.
24. **Scenario:** Distress is accompanied by fever and unusual sleepiness. What is the priority?
A) Routine observation only. B) Prompt professional assessment. C) Argument. D) Automatic sedation.
Answer: B. LO: 7, 10.
25. **Scenario:** A client repeatedly walks to the gate at 17:00. What should the team do?
A) Lock the client in a room. B) Explore purpose, review hazards and develop an individualised safe movement plan. C) Remove shoes. D) Threaten punishment.
Answer: B. LO: 6–7.
26. **MCQ:** A missing person with dementia requires: A) delayed reporting; B) immediate use of the emergency pathway; C) private investigation only; D) blame. **Answer: B. LO: 7.**
27. **Scenario:** A person falls twice at dusk. What should the caregiver do?
A) Dismiss it as sundowning. B) Report the falls and change, protect safety and seek review. C) Alter medication. D) Restrain.
Answer: B. LO: 7, 9.

28. **True/False:** Sudden inability to stand can be attributed automatically to dementia.
Answer: False. LO: 7.
29. **Scenario:** New hallucinations, fever and confusion appear. What should the caregiver do?
A) Diagnose. B) Escalate urgently and record signs and timing. C) Wait until morning. D) Argue.
Answer: B. LO: 7, 10.
30. **Application:** Name four safety or sudden-change signs requiring escalation. **Answer:** Sudden confusion, weakness, fever, new fall, severe pain, breathing difficulty, unusual drowsiness, rapid speech change or inability to walk. **LO: 7.**
31. **Scenario:** A family says “he is aggressive,” but distress occurs only during shaving. What should the caregiver do?
A) Accept the label. B) Describe the task-specific pattern and review approach, privacy, pain and communication. C) Punish. D) Stop all care.
Answer: B. LO: 8–9.
32. **MCQ:** Objective documentation says: A) “She was difficult” ; B) “She pushed the towel away and said ‘no’ ” ; C) “She wanted attention” ; D) “She is aggressive.”
Answer: B. LO: 9.
33. **Scenario:** A sudden change is placed only in a form and the supervisor has not seen it. What should happen?
A) Assume receipt. B) Make direct authorised escalation and document it. C) Tell neighbours. D) Wait.
Answer: B. LO: 7, 9.
34. **What should the caregiver NOT do?** Diagnose, prescribe, independently change the care plan, investigate allegations or initiate restraint outside authorised procedure.
Answer: These actions exceed scope. **LO: 10.**
35. **Integrated scenario:** A client refuses dinner, tries to leave, appears newly confused and a relative asks for restraint and secrecy. List the professional sequence.
Answer: Pause; observe; consider the person and possible triggers; communicate calmly; reduce pressure and adapt; protect dignity and immediate safety; escalate the sudden change and risk promptly; document facts, response and notifications; and maintain confidentiality. **LO: 1–10.**

17. Answer key and explanations

Q	Answer	Reasoning	LO
1	B	Variation and context matter.	1
2	B	Participation supports autonomy.	2, 6
3	B	Personhood is broader than diagnosis.	1–3
4	False	Dementia does not automatically remove choice.	3
5	Include support	Meaningful activity should be preserved.	2, 6
6	Do not infantilise	Adult dignity must be protected.	3
7	B	Pace and sensory support improve communication.	2
8	B	Explore emotion and purpose.	2, 5
9	B	Reassurance is not argument or routine deception.	5
10	B	Keep the person central.	8
11	False	Culture does not remove safety or dignity duties.	8
12	B	New disorientation requires observation and escalation.	5, 7
13	B	Behaviour may communicate a need or response.	3
14	B	Look for physical and contextual contributors.	3–4

15	B	Safe meaningful activity can support wellbeing.	3, 6
16	False	Labels do not provide useful evidence.	9
17	B	Repetition may express anxiety or loneliness.	3–4
18	Context plus behaviour and outcome	Patterns support review.	9
19	B	Reduce stimulation and threat.	5
20	B	Restore privacy and adapt care.	5–6
21	A	Resistance has many possible contributors.	4–5
22	False	Medication decisions are outside caregiver scope.	10
23	B	De-escalate and seek authorised help.	5, 7
24	B	Acute signs require review.	7, 10
25	B	Preserve movement while managing risk.	6–7
26	B	Missing-person risks require urgent action.	7
27	B	Falls and change are safety concerns.	7, 9
28	False	Sudden functional change may have an acute cause.	7
29	B	Escalation is safer than diagnosis or delay.	7, 10

30	Multiple signs	Red flags require professional review.	7
31	B	Task-specific patterns guide adaptation.	8–9
32	B	It records observable facts.	9
33	B	Urgent concerns need direct confirmation.	7, 9
34	Do not do these	They exceed caregiver scope.	10
35	P-O-C-T-A-P-D-E	Integrates the professional response framework.	1–10

18. Recommended behavioural competency verification

Online assessment demonstrates knowledge and scenario reasoning; it does not prove that the learner will always communicate calmly or behave appropriately in practice. NIC or an employer should use a separate verification activity when observed competence is required.

Method	What it tests	Evidence
Communication role-play	Introduction, pace, listening, reassurance and choice	Assessor rubric
Resistance-to-care scenario	Privacy, consent, adaptation and de-escalation	Observation checklist
Distress simulation	Trigger recognition, calm approach and escalation	Assessor notes
Wandering/safety scenario	Least restrictive safety planning and urgent response	Structured decision record
Family conversation	Objective communication, partnership and confidentiality	Role-play outcome
Documentation exercise	Trigger, behaviour, response, outcome and escalation record	Marked sample
Supervisor observation	Real workplace conduct, dignity and consistency	Signed verification form

High-level competency checklist

The learner should be able to:

1. approach calmly and safely;
2. use a respectful tone, simple language and appropriate pace;
3. listen actively and respond to emotion;
4. recognise the person's preferences, strengths and communication signals;
5. protect dignity, privacy and choice;
6. identify possible triggers without diagnosing;
7. de-escalate distress and resistance without force or humiliation;
8. recognise wandering, falls, sudden change and immediate danger;
9. document objective observations and effective responses;
10. escalate appropriately and maintain confidentiality.

A critical fail should include shouting or humiliating the person, unnecessary force, unauthorised restraint, diagnosing or prescribing, concealing sudden deterioration, falsifying a record or sharing confidential information.

19. Completion requirements

NIC should issue the credential when the learner has completed all 18 lessons, attempted the six module assessments, achieved at least **80% (28/35)** on the final assessment and acknowledged the scope statement. A failed final assessment may be retaken after review; NIC may permit up to three attempts before supervisor review or re-enrolment.

Behavioural verification should be required where an employer needs evidence of observed communication, de-escalation, resistance-to-care response, documentation or safety judgement. It should be recorded separately from the online certificate.

NIC should retain the learner's identity, enrolment and completion dates, curriculum version, lesson and module completion, final score and attempts, certificate identifier, scope acknowledgement and practical-verification outcome where applicable. The credential demonstrates completion of structured professional development and scenario-based assessment. It does not prove diagnostic competence, clinical competence, permanent behavioural competence or specialist dementia-care status.

20. Certificate wording

Certificate title

NIC CPD Micro-Credential: Dementia Care, Communication & Behaviour Support

Competency statement

This credential recognises successful completion of NIC continuing professional development in dementia care, communication and behaviour support. The learner demonstrated knowledge-based and scenario-based understanding of person-centred dementia care, communication adaptation, behaviour as communication, distress, resistance to care, meaningful engagement, safety, wandering, sudden change, family partnership, documentation and escalation within caregiver scope of practice.

The certificate should state the 5–6 CPD learning hours, completion date, assessment pass mark, certificate identifier and curriculum version. It should include: **“This CPD micro-credential is not a diagnosis, clinical qualification, psychotherapy credential, specialist neurological-care qualification or regulated licence. It does not make the holder a nurse, doctor, clinician or dementia specialist unless separately qualified.”**

21. Digital badge specification

Field	Recommended value
Badge name	NIC CPD Dementia Care, Communication & Behaviour Support
Description	Recognises completion of NIC CPD focused on personhood, dementia communication, behaviour as communication, distress support, resistance to care, meaningful engagement, safety, wandering, sudden change, family partnership, documentation and escalation.
Issuer	National Institute of Caregivers Nigeria
CPD hours	5–6 learning hours
Level	Professional CPD; intermediate workplace application
Competencies represented	Person-centred care; communication; trigger recognition; behaviour support; dignity; independence; safety; family partnership; documentation; escalation

Evidence requirements	18 lessons, six module assessments, 35-question final assessment at 80% and scope acknowledgement; behavioural verification where separately required
Verification information	NIC verification URL or QR code, certificate ID, completion date and curriculum version
Limitation	Not a clinical licence or specialist dementia qualification
Review period	Recommend review at least every 24 months, or sooner when relevant evidence, local policy or reporting routes change

22. NIC portal course description

Dementia Care, Communication & Behaviour Support is a 5–6 hour NIC CPD micro-credential for caregivers and direct-care professionals who already understand the fundamentals of caregiving. It develops the professional judgement needed to support people living with dementia with dignity, patience, safety and respect.

Through realistic home-care, residential-care, community and healthcare-support scenarios, learners practise communication, reassurance, confusion and disorientation support, behaviour-as-communication, repetitive behaviour, agitation, resistance to personal care, wandering, time-of-day changes, sudden deterioration, family communication, documentation and escalation.

The course teaches a practical sequence: **pause, observe, consider the person, identify possible triggers, communicate calmly, adapt, protect dignity and safety, document and escalate when necessary**. It does not make learners clinicians, diagnosticians, psychotherapists, investigators or specialist dementia professionals.

23. References

- [1] World Health Organization, Dementia, 3 July 2026.
- [2] World Health Organization, Towards a dementia plan: a WHO guide.
- [3] National Institute for Health and Care Excellence, Dementia: assessment, management and support for people living with dementia and their carers, NG97, published 2018 and reviewed 2025.
- [4] Alzheimer's Society, Communicating and dementia.
- [5] National Institute for Health and Care Excellence, Dementia Quality Standard QS184, Quality statement 6: Managing distress.

[6] World Health Organization, iSupport for dementia.

[7] Alzheimer's Association, Wandering.

Source-use statement: WHO, NICE and Alzheimer's Society resources are used as international or evidence-informed practice references. They do not constitute Nigerian law or NIC accreditation requirements. NIC and employers should verify current Nigerian clinical, safeguarding, emergency, privacy and reporting arrangements before publication and insert local contacts into the portal.

24. Quality assurance review

Quality question	Finding	Implementation action
CPD value	The course concentrates on dementia-specific judgement, communication, distress and safety rather than basic caregiving.	Keep foundational prerequisites visible.
Non-duplication	General ageing and basic personal care are not retaught in depth.	Cross-link NIC Elderly Care & Gerontology instead of duplicating it.
Person-centred care	Every module reinforces personhood, strengths, preferences, dignity and remaining abilities.	Review all portal wording for labels such as “difficult” or “behaviour problem.”
Communication	Practical pace, language, non-verbal communication, reassurance and family strategies are taught.	Include role-play where possible.
Behaviour support	Behaviour is treated as potential communication and trigger mapping is taught.	Require objective observation in assessment.
Distress and resistance	De-escalation, choice, timing, privacy and adaptation are covered.	Add employer-specific escalation routes.
Safeguarding	Abuse, neglect, inappropriate restraint, sudden change and concealment risks are addressed.	Add current local safeguarding contacts.

Safety	Wandering, getting lost, falls, aggression and acute change are covered.	Provide emergency procedure links for each setting.
Scope	Learners are told not to diagnose, prescribe, investigate or restrain outside authorisation.	Make scope acknowledgement mandatory.
Cultural relevance	Family networks, language, faith, household routines, traffic, heat, open drains and power/noise contexts are acknowledged.	Review examples with Nigerian dementia-care educators.
Assessment quality	29 of 35 final items are scenario/application based.	Retain explanations, not answer letters alone.
Practical value	Strategies can be applied immediately in home, residential, community and healthcare-support settings.	Add local case variations and supervisor exercises.
Professional credibility	The course includes competency framework, structured assessment, verification, certificate limitations and review period.	Obtain formal NIC subject-matter and governance approval.
Evidence integrity	External claims are linked to traceable WHO, NICE and Alzheimer's Society sources.	Recheck links and guidance before each major revision.

Final implementation checklist

Before launch, NIC should confirm that the portal contains the sequence **COURSE → MODULES → LESSONS → MODULE ASSESSMENTS → FINAL ASSESSMENT → CERTIFICATE**. Every lesson should display its objectives, scenario, professional risks, key practice points and assessment. The portal should provide a downloadable person-centred observation template and a local escalation directory.

Each employer should insert current contacts for the supervisor, clinical lead, emergency service, safeguarding lead, complaints route and alternative escalation route when the normal supervisor is involved. The course should also be reviewed for accessibility, language, trauma-informed presentation, data protection, secure assessment handling and local cultural fit.

The intended NIC professional standard is:

The learner sees the person, understands that behaviour may communicate a need, responds calmly and respectfully, looks for possible triggers, adapts the approach, protects dignity and safety, and knows when a change or situation requires documentation and escalation.